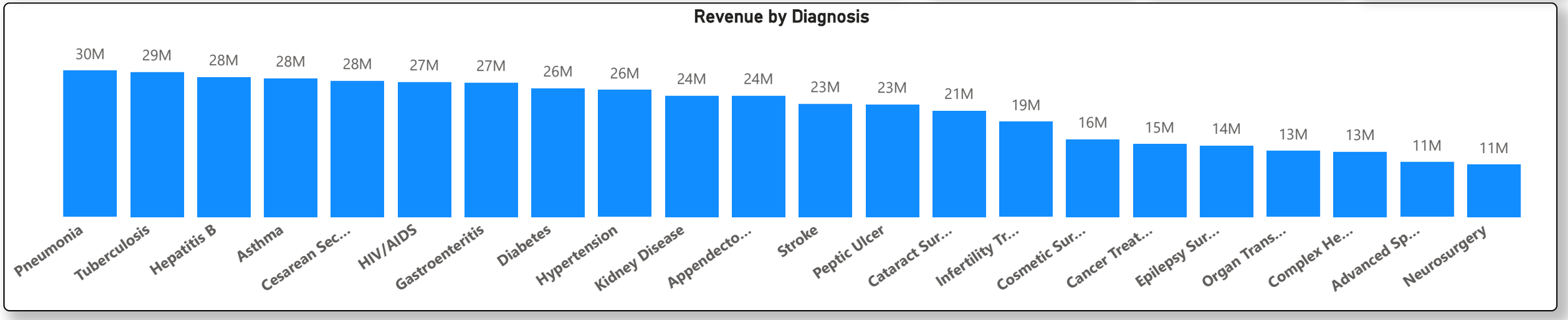
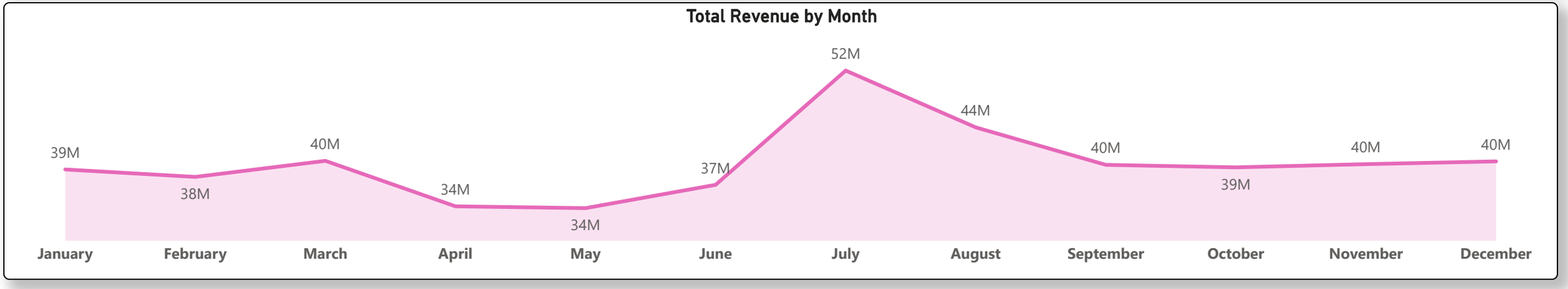
Healthcare Claims Analytics Report

The system processed 1,500 claims totaling 476.48M, with a mid-year peak in July (52M). Fraud represents 40.87% of claims but contributes a disproportionately higher 52.06% of total billed amount, indicating that high-value fraudulent claims are a key driver of overall costs and should be prioritized for investigation.

Number of Claims	Total Amount Billed	Fraud Amount
1500	476.48M	248.08M

Average Claim Value	Fraud Rate %	Non Fraud Amount
317.66K	40.87	228.40M





Fraud Analysis

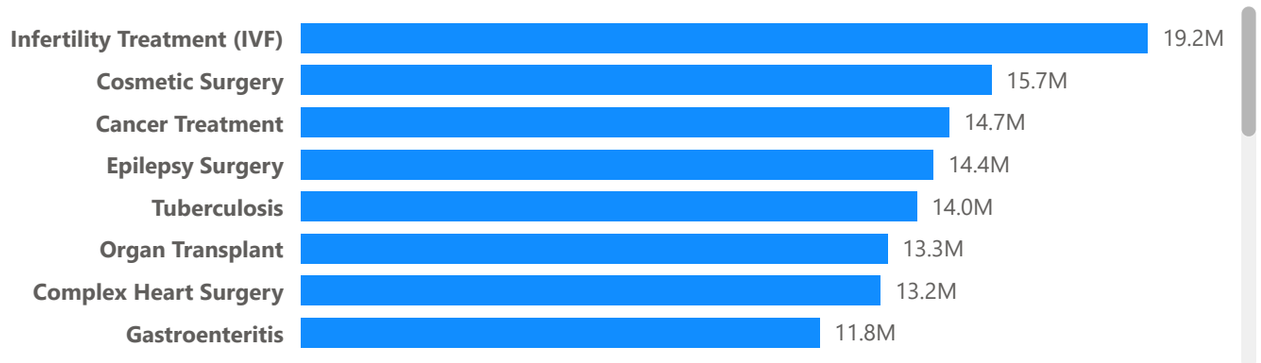
Fraud claims make up 40.87% of cases but contribute 52.06% of total revenue, highlighting high-value fraud concentrated in specific treatments like IVF and Cosmetic Surgery.

Gender
- Female
- Male

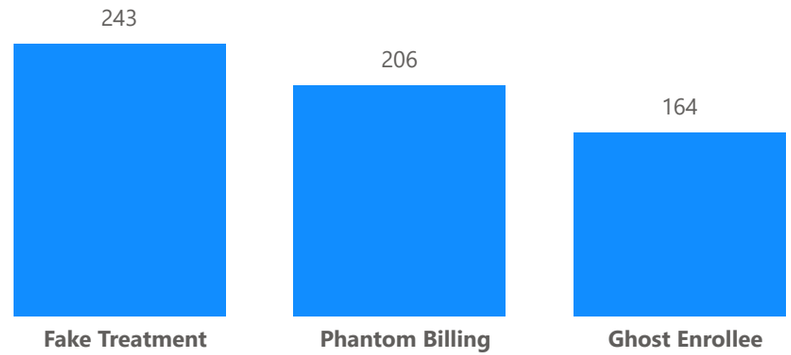
Fraud Flag
- No
- Yes

Fraud Cases	Fraud Amount	Fraud Revenue %	Fraud Claim %	Average Fraud Claim
613	248.08M	52.06	40.87	404.70K

Fraud Amount by Diagnosis

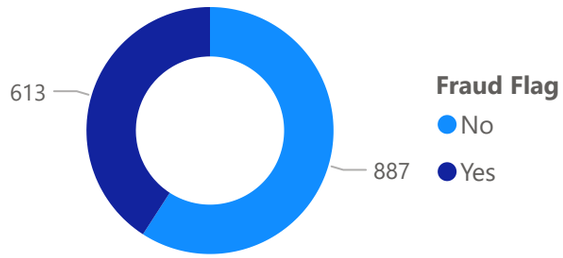


Fraud Cases by Fraud Type

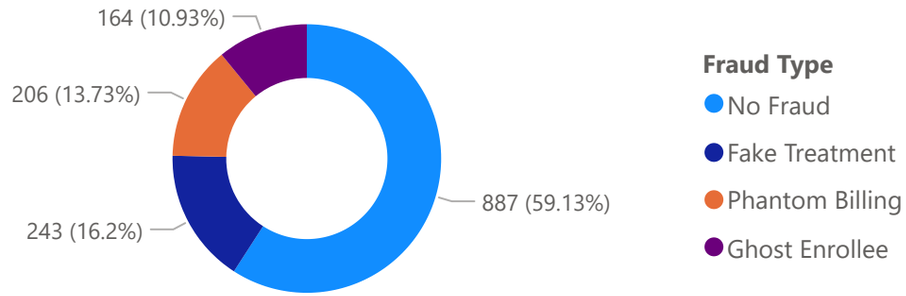


Fraud Amount by Treatment

Patients by Fraud Flag



Total Claims by Fraud Type

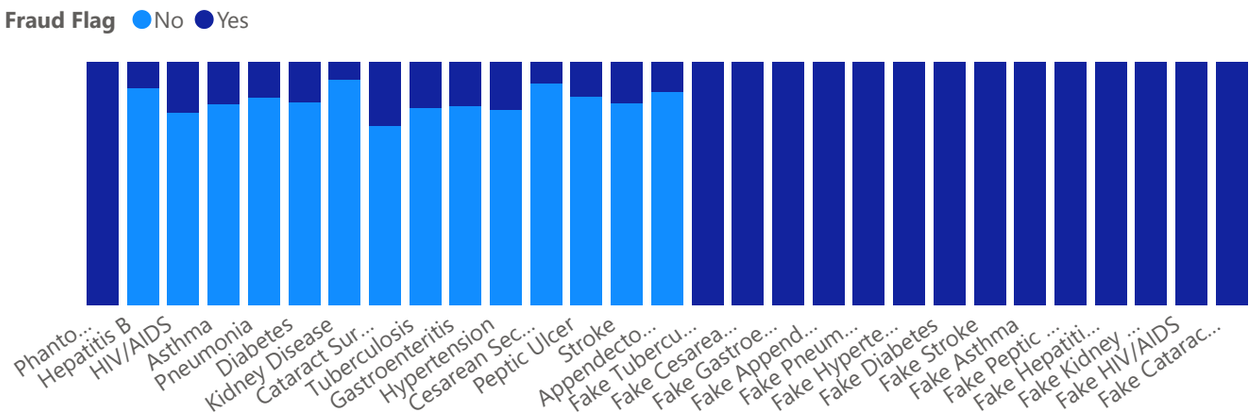


Cost and Operations Analysis

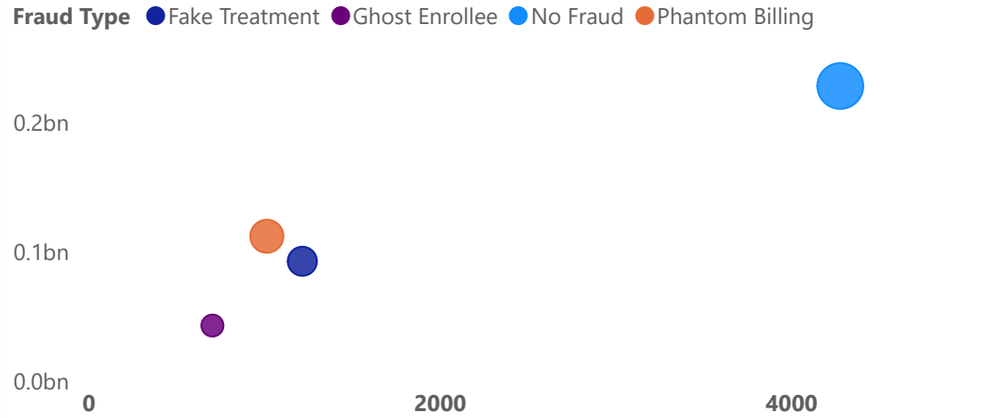
High-cost treatments and shorter stays show elevated cost per day, while fraud-linked procedures drive significant billing, highlighting inefficiencies and targeted cost leakage areas.

Total Hospital Days	Average Length of Stay	Average Cost per Day
7229	4.82	84.99K

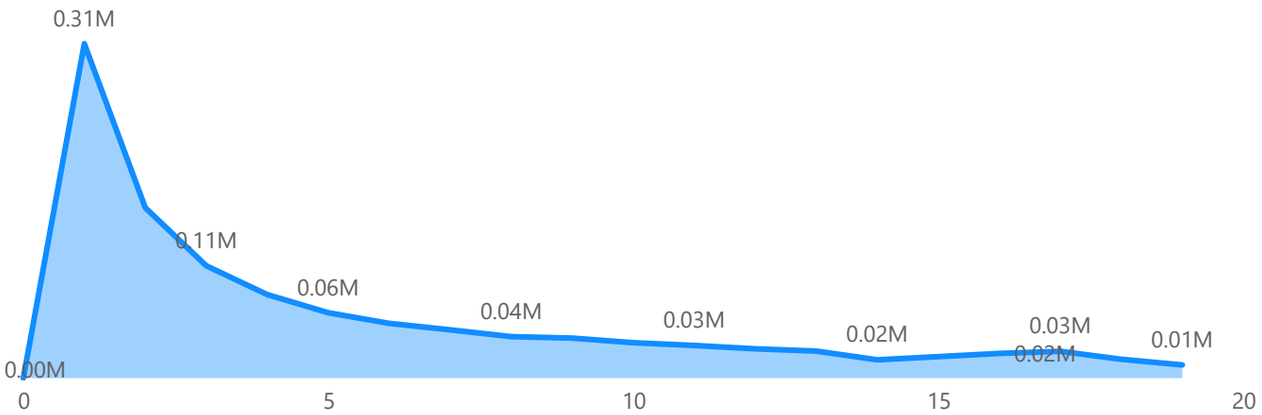
Amount Billed by Treatment and Fraud Flag



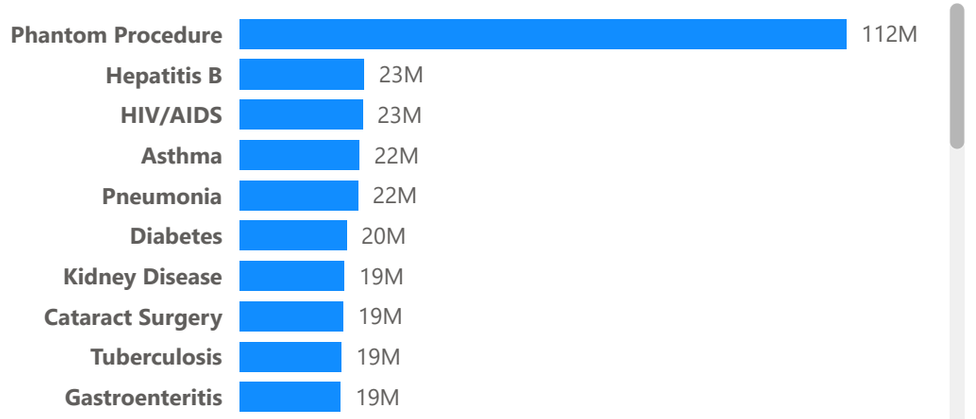
Cost vs Length of Stay



Average Cost per Day by Length of Stay



Cost by Treatment



Insights

This analysis covers 1,500 healthcare claims with a **total billed amount of 476.48M**, highlighting key cost drivers, fraud patterns, and operational inefficiencies.

A significant finding is that **fraud accounts for 40.87% of claims but contributes a higher 52.06% of total billed amount**, indicating that fraudulent claims are fewer in number but higher in value.

Average fraud claim (404.7K) exceeds overall claim average (371.66K) showcasing significantly high financial impact.

Costs are concentrated in a few diagnoses such as Pneumonia, Tuberculosis, and Hepatitis B, while certain procedures like Phantom Procedures drive exceptionally high spending.

A mid-year spike in July suggests possible seasonal or operational factors influencing claim volumes.

From an operational perspective, the **average length of stay is 4.82 days**, with higher cost per day observed in shorter stays, indicating potential inefficiencies or intensive billing practices.

A relatively small proportion of high-value fraudulent and high-cost claims is driving a significant share of total healthcare expenditure and targeted intervention in these areas can deliver substantial cost savings and risk reduction.

Overall, the analysis suggests that focusing on high-cost treatments and high-value fraud cases can significantly reduce financial leakage and improve cost efficiency.